

Pelvic exenteration for women and people assigned female at birth

Pelvic exenteration in women and people assigned female at birth is an operation (surgery) to remove multiple organs in the pelvis.

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About pelvic exenteration

Pelvic exenteration is a major operation to remove multiple organs in the pelvis. It can be used to treat cancer in the pelvis. The pelvis is the area of the body between the hips, in the lower part of the tummy (abdomen). It contains:

- the sex organs
- the bladder
- a section of the small bowel
- the lower end of the large bowel (colon, rectum and anus).

The pelvis also contains the pelvic bones, lymph nodes (glands), blood vessels and nerves.

For women, trans men and other people assigned female at birth, the sex organs you are born with include the:

- ovaries
- fallopian tubes
- uterus (womb)
- cervix
- vagina.

When is pelvic exenteration used?

Pelvic exenteration is sometimes used to treat cancer that has spread within the pelvis, or has come back in the pelvis after other treatment.

It may be used to treat:

- cervical cancer
- womb cancer
- vaginal cancer
- rectal cancer
- anal cancer
- bladder cancer
- some vulval cancers.

We have more information about treatment for your [type of cancer](#) and what happens [before and after the operation](#).

Types of pelvic exenteration

There are different types of pelvic exenteration:

- anterior exenteration
- posterior exenteration
- total exenteration
- other exenteration operations.

Your surgeon decides on the type of exenteration you need, depending on the results of the tests and scans you have had. They will discuss these at a multi-disciplinary team (MDT) meeting. It also depends on:

- the [type of cancer](#) you have
- where the cancer is in the pelvis
- what treatments you have had before.

If you are transgender or non-binary

If you have had genital gender-affirming surgery, you may have additional questions about how this affects the treatment you need. Your doctor or nurse can give you more information about what your operation will involve.

We also have more information for [trans and non-binary people](#) about cancer, cancer treatment and finding support, that you may find helpful.

Anterior exenteration

This operation involves removing organs in the front part of the pelvis. The surgeon will remove:

- the bladder
- the cervix
- part or all of the vagina
- the womb
- the ovaries if necessary.

Image: Anterior exenteration

Posterior exenteration

This operation involves removing organs in the back part of the pelvis. The surgeon will remove:

- the cervix
- the womb
- part or all of the vagina
- the lower part of the bowel – the rectum and sometimes the anus
- the ovaries if necessary.

Image: Posterior exenteration

Total exenteration

This surgery involves removing all the organs in the pelvis. The surgeon will remove:

- the womb
- the cervix
- part or all of the vagina
- the bladder
- the lower part of the bowel - the rectum and sometimes the anus
- the ovaries if necessary.

Image: Total pelvic exenteration

Other exenteration operations

Sometimes your surgeon may suggest an operation that involves taking more areas of tissue. For example, as part of the operation they may need to remove:

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muscles

- nerves
- bones
- blood vessels.

If you need more extensive surgery like this, your surgeon will explain what is involved.

If you are having reconstruction

Depending on what type of pelvic exenteration you have, parts of the body may need to be reconstructed. This means having plastic surgery to rebuild parts of the body that have been removed.

To do this the surgeon may use flaps of skin, fat and muscle. These are taken from other parts of the body such as the buttocks, thighs or tummy. These flaps are shaped and stitched into place. This may be done at the same time as your pelvic exenteration operation, or at a later date.

You will have a wound where these flaps are taken from. A nurse will look after this when you are in hospital and will arrange for it to be looked after when you get home.

If part or all of the vagina is removed, sometimes the surgeon can make a new one. This is called [vaginal reconstruction](#).

If the bladder or rectum are removed

If the bladder or rectum are removed during your surgery, the surgeon will make a new way for urine (pee) or stools (poo) to leave the body. Your surgeon or nurse will talk to you about this before surgery. It usually means you will have a stoma after surgery.

A stoma is an opening that the surgeon makes on your tummy (abdomen). You may have a stoma that:

- allows stools to pass out of the body, called a [colostomy](#)
- allows urine to pass out of the body, called a [urostomy](#)
- allows urine and stools to pass out of the body, called a Carter stoma or double barrel stoma.

You wear a disposable bag over the stoma to collect the stools or urine.

Sex life after having pelvic exenteration

The physical changes to your body after surgery can mean changes to your sex life. The operation will usually affect you both physically and emotionally. Your surgeon and specialist nurse will talk to you about the changes you may have. It can help to talk to them about any questions or worries you have.

We have more information about:

- [cancer and your sex life](#) – including tips on talking to your healthcare team and coping with your feelings

- [female pelvic changes and sex](#) – this includes changes in sensation, bladder problems and changes to the anus or rectum
- [sex and side effects of cancer treatment](#) – including advice on coping with a stoma
- [body image](#) – if you are worried about changes to your appearance.

You may not feel like having sex for a while. If you have a partner, you may both need time to get used to any changes. There are different ways you can show your partner you care about them.

This can include spending time together and showing affection through touching, holding hands or putting an arm around their shoulder. You may also find new ways to share sexual pleasure.

After surgery to remove the vagina, you will not be able to have penetrative sex. If the clitoris was not removed, you may still have sensations and be able to orgasm from this area. But during the operation the nerves may have been damaged, which means this area may feel different.

After treatment, you may find that your sex life gradually improves. If you continue to have difficulties, ask your specialist nurse or doctor for advice. They can refer you for more specialised support if needed.

Fertility after pelvic exenteration

This type of surgery removes the parts of the body needed to get pregnant. For some people, this can be difficult to cope with. It is important to talk to someone you trust, or you may find it easier to talk to a counsellor. Your GP can refer you.

Macmillan is also here to support you. If you would like to talk, you can:

- Call the Macmillan Support Line for free on [0808 808 00 00](tel:0808 808 00 00).
- [Chat to our specialists online](#)
- Visit our [Online Community](#) to talk to people who have been affected by cancer, share your experience, and ask an expert your questions.

If you have not been through the menopause, it will start after pelvic exenteration. Your doctor or a menopause specialist should support you. They will talk to you about how to [manage menopausal symptoms](#).

You can ask to be referred to a fertility specialist before having cancer treatment. You may be able to store your eggs or embryos. These are fertilised eggs.

If you decide you want to try and have a child in the future, you may be able to use a surrogate. A surrogate is someone who may be able to carry the child for you in their womb. Or you may want to consider adoption in the future.

We have more information about [fertility in women](#).

Your feelings after pelvic exenteration

Having pelvic exenteration surgery can be difficult both physically and emotionally. You may have different feelings like [stress, anger, anxiety and fear](#). There is no right or wrong way to feel. How you feel is individual to you.

It is important that you take time to process how you are feeling. Talking to someone you trust may help. Sometimes it is easier to talk to someone who does not know you. You could ask your cancer doctor or GP to refer you to a counsellor.

You may also find it difficult to cope with needing help from others while you are recovering. These are all normal feelings. They are part of adjusting.

Where to get help and support

As well as the support available from Macmillan, there are other organisations that can offer you practical and emotional support.

Colostomy UK



[Colostomy UK](#) provides practical help, support and reassurance to anyone who has had, or is about to have, a colostomy. It also offers support for carers, friends, and family members.

IA – Ileostomy and Internal Pouch Association



[IA](#) offers support to help anyone who has had, or is about to have, their colon removed and has an ileostomy or internal pouch.

Urostomy Association



[Urostomy Association](#) offers support and advice to anyone who has had, or is about to have, surgery resulting in a urostomy. It also offers support for family members and carers.

Eve Appeal



[Eve Appeal](#) supports people affected by gynaecological cancers, and their family members and friends.

Bowel Cancer UK



[Bowel Cancer UK](#) offers information and support to people affected by bowel cancer.

Anal Cancer Foundation



[Anal Cancer Foundation](#) provides support and information for people having treatment for anal cancer, and their carers and family members.

LGBT Foundation



[LGBT Foundation](#) provides advice, support and information to people who are gay, lesbian, bisexual or transgender, and their families and friends.

Live Through This



[Live Through This](#) run a peer support group for LGBTIQ+ people affected by cancer. This group focuses on LGBTIQ+ experiences of cancer and support, rather than on any one cancer type.

Stonewall



[Stonewall](#) provides information and support to the LGBT community.

College of Sexual and Relationship Therapists



[College of Sexual and Relationship Therapists](#) has a directory of sex and relationship therapists.

Human Fertilisation and Embryology Authority



[Human Fertilisation and Embryology Authority](#) provides free impartial information to people affected by fertility treatment. It has information about choosing a clinic.

Fertility Friends



[Fertility Friends](#) is an online community for people coping with infertility.

Fertility Network UK



[Fertility Network UK](#) is an online community for people coping with infertility or adoption.

Daisy Network



[Daisy Network](#) provides information and support to people with premature menopause, including information on treatment and research and an online network.

About our information

References



Below is a sample of the sources used in our pelvic exenteration information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

The European Society for Medical Oncology. Cervical cancer: ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up. 2017. Available from [www.annalsofoncology.org/article/S0923-7534\(19\)42148-0/pdf](http://www.annalsofoncology.org/article/S0923-7534(19)42148-0/pdf) (accessed November 2022).

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Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Chief Medical Editor, [Professor Tim Iveson](#), Consultant Medical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

Date reviewed

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help